

Obesity Patient Panel



Janine V. Kyrillos, MD FACP

Diplomate of the American Board of Obesity Medicine

Associate Professor of Medicine,

Sidney Kimmel Medical College at Thomas Jefferson University

Director, Jefferson Comprehensive Weight Management

Disclosure

- NovoNordisk: Speaker's Bureau, Advisory Board
- Lilly: Advisory Board

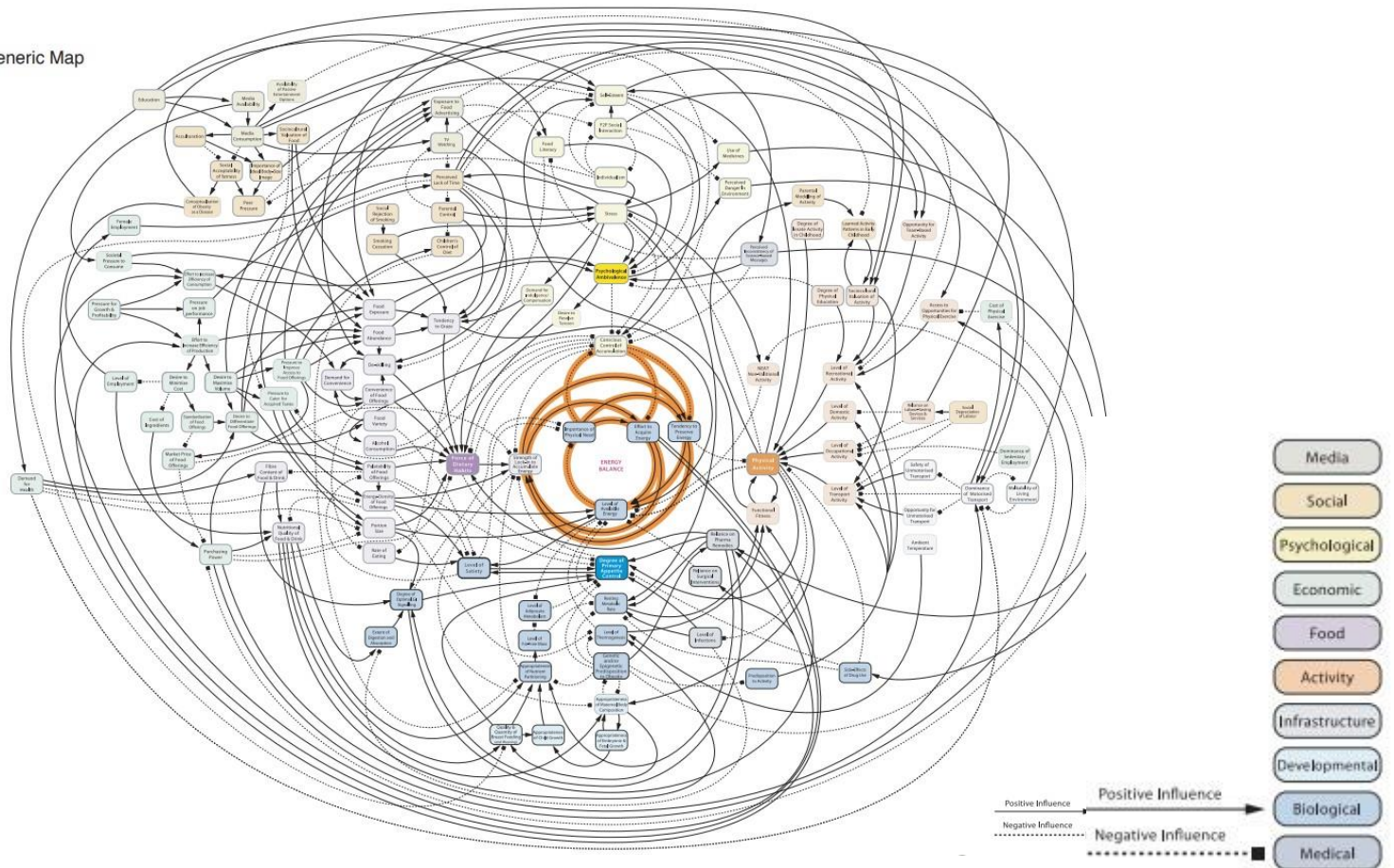
Objectives

- Identify obesity as chronic disease with multiple contributing factors
- Recognize treatment goals incorporating a weight inclusive approach
- Describe how bias and stigma affect people living with obesity
- Utilize person first and patient-centered language
- Interact with persons with obesity with empathy and compassion

Contributors to Excess Weight

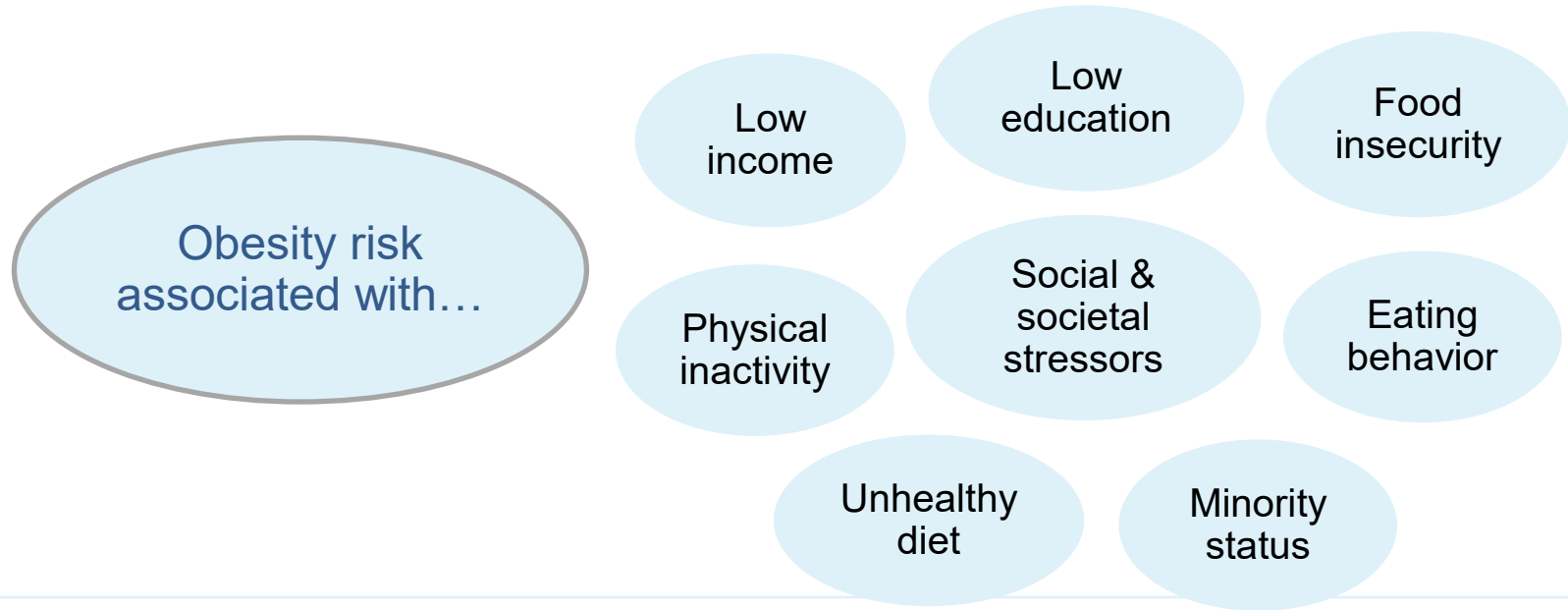
Map 0

Full Generic Map



Obesity and social determinants of health

There is persistent substantial disparities among certain population groups, all of which further complicate the efforts to understand, control, and prevent obesity



U.S. Centers for Disease Control and Prevention. www.cdc.gov/obesity/index.html
May AL, et al. MMWR Morbidity and Mortality Weekly Report. 2013;62(3 Supplement):118-128.

Without treatment, the odds are brutal

1 in 1,290
men

1 in 677
women

Odds of reaching a “normal” weight from class III obesity — without medication or surgery

Evidence Based Treatment



Surgical Procedures
(BMI ≥ 40 or ≥ 35 with related complications)

Medication
(BMI ≥ 30 or ≥ 27 with related complications)

Lifestyle Changes
(all BMIs)

FDA Approved Medications

- Liraglutide 3mg (Saxenda)
- Naltrexone-bupropion (Contrave)
- Orlistat (Xenical)
- Phentermine-topiramate (Qsymia)
- Semaglutide 2.4 (Wegovy)
- Super Absorbent Hydrogel Particles (Plenity)
- Sympathomimetic amines
 - Phentermine
 - Phendimetrazine
 - Diethylpropion

Surgical Procedures

- Sleeve Gastrectomy
- Roux-en-Y Gastric Bypass
- Adjustable Gastric Band
- Biliopancreatic Diversion with Duodenal Switch
- Single Anastomosis Duodeno-Ileal Bypass with Sleeve Gastrectomy
- Endoscopic procedures

Off Label Medications

- Bupropion
- GLP-1 Agents
- Metformin
- Naltrexone
- Tirzepatide
- Topiramate
- Vyvanse

Weight Normative Approach

Weight and BMI

- Magic Numbers
- Unrealistic Expectations
- Learned Helplessness
- Weight Cycling
- Shame
- Eating Disorders

Weight Inclusive Approach

Health and Wellbeing

- Quality of Life
- Lower morbidity
- Energy
- Decrease Risk
- Improved Mobility

Weight is only one data point!

Weight Bias and Stigma

The New York Times

Why Do Obese Patients Get Worse Care? Many Doctors Don't See Past the Fat

What It's Like to Go to the Doctor When You're Fat

"From the start of my life, I've been told I'm too fat."

EXPLORE Health

HOME > WEIGHT LOSS

Are You Afraid to Go to the Doctor Because You're Fat?

Does stepping on the scale in front of your doc give you a panic attack? Learn how to fight size discrimination in the doctor's office.

Identity

How to Find a Fat-Positive Doctor Who Won't Just Tell You to Lose Weight

In many doctors' opinions, weight loss is the only treatment for fat patients regardless of their health concerns, as though the answers to our problems are written all over our bodies. But I have the right to be fat and healthy.

by Charlotte Zoller

SELF

Fitness Food Health Love Beauty Culture

Health | June 26, 2018

Weight Stigma Kept Me Out Of Doctors' Offices for Almost a Decade

Please believe patients when they say fat bias happens.

By Nina Goldstein

Hospital
son T

BODY WORK

Johanna Youngmans

September 12, 2019 11:41 AM

"Fat Shaming" Won't Solve Obesity. Science Might.

A round up of 15 research papers and articles from Michigan Medicine proving why the treatment for obesity can't adopt a "one size fits all" approach.

EXPLORE SHAPE

SHAPE.COM > LIFESTYLE > MIND AND BODY

I Was Fat Shamed By My Doctor and Now I'm Hesitant to Go Back

Sizeism in the medical community, which can lead doctors to fixate on a patient's weight, doesn't cause psychological trauma, but potentially misdiagnosis. Here, one woman shares her story of what it's like to feel bullied by your own doc.



Jefferson Health



Attitudes of Healthcare Providers

Patients with obesity stereotyped as being:

Non-compliant
Lazy
Lacking in self-control
Awkward
Weak-willed
Sloppy
Unsuccessful
Unintelligent
Dishonest

Physicians
Physician Assistants
Nurses
Dietitians
Psychologists
Fitness Professionals

Students in medicine,
nursing, occupational
therapy, psychology

Berryman et al., 2006; Brown, 2007; Ferrante et al., 2009; Hebl & Xu, 2001; Huizinga et al., 2009, 2010; Miller et al., 2013; Pantenburg et al., 2012; Phelan et al., 2014; Puhl et al., 2013, 2014; Swift et al., 2013; Vroman & Cote, 2011.

Actions of healthcare providers

- Less time in appointments
- Less discussion with patients
- Less intervention
- Less respect to patients with higher weight

Amy et al., 2006; Gudzone et al., 2013; 2014; Mensinger et al., 2018;
Mulherin et al., 2013; Phelan et al., 2015; Turner et al., 2012



Obesity Stigma Affect on Access to Care

- Less likely to seek preventive care screening⁴
- More likely to doctor shop⁶
- 80% more likely to visit the emergency room... but no more likely to be hospitalized⁶

1. Puhl RM, Brownell KD. Confronting and coping with weight stigma: an investigation of overweight and obese adults. *Obesity* (Silver Spring) 2006;14(10):1802-1815 [PubMed]

2. Puhl RM, Moss-Racusin CA, Schwartz MB. Internalization of weight bias: implications for binge eating and emotional well-being. *Obesity* (Silver Spring) 2007;15(1):19-23 [PubMed]

3. Vartanian LR, Shaprow JG. Effects of weight stigma on exercise motivation and behavior: a preliminary investigation among college-aged females. *J Health Psychol* 2008;13(1):131-138 [PubMed]

4. Amy NK, Aalborg A, Lyons P, Keranen L. Barriers to routine gynecological cancer screening for White and African-American obese women. *Int J Obes (Lond)* 2006;30:147-155. [PubMed]

5. Gudzone KA, Bennett WL, Cooper LA, Bleich SN. Perceived judgment about weight can negatively influence weight loss: a cross-sectional study of overweight and obese patients. *Prev Med.* 2014;62:103-107. [PMC free article] [PubMed]

6. Gudzone, K.A., Bleich, S.N., Richards, T.M., Weiner, J.P., Hodges, K. and Clark, J.M. (2013), Doctor shopping by overweight and obese patients is associated with increased healthcare utilization. *Obesity*, 21: 1328-1334. <https://doi.org/10.1002/oby>.

Behavioral Consequences

- 79% reported coping on multiple occasions by eating more food¹
- More frequent binge eating and refusal to diet²
- More likely to avoid exercise ³
- If person feels judged, they are less likely to lose weight⁵

1. Puhl RM, Brownell KD. Confronting and coping with weight stigma: an investigation of overweight and obese adults. *Obesity (Silver Spring)* 2006;14(10):1802-1815 [PubMed]

2. Puhl RM, Moss-Racusin CA, Schwartz MB. Internalization of weight bias: implications for binge eating and emotional well-being. *Obesity (Silver Spring)* 2007;15(1):19-23 [PubMed]

3. Vartanian LR, Shaprow JG. Effects of weight stigma on exercise motivation and behavior: a preliminary investigation among college-aged females. *J Health Psychol* 2008;13(1):131-138 [PubMed]

4. Amy NK, Aalborg A, Lyons P, Keranen L. Barriers to routine gynecological cancer screening for White and African-American obese women. *Int J Obes (Lond)* 2006;30:147-155. [PubMed]

5. Gudzone KA, Bennett WL, Cooper LA, Bleich SN. Perceived judgment about weight can negatively influence weight loss: a cross-sectional study of overweight and obese patients. *Prev Med*. 2014;62:103-107. [PMC free article] [PubMed]

6. Gudzone, K.A., Bleich, S.N., Richards, T.M., Weiner, J.P., Hodges, K. and Clark, J.M. (2013), Doctor shopping by overweight and obese patients is associated with increased healthcare utilization. *Obesity*, 21: 1328-1334. <https://doi.org/10.1002/oby>.



Consequences of obesity bias

- Psychological
 - Anxiety, depression, low self esteem, eating disorder, substance abuse, suicide
- Physiological
 - Increase in blood pressure, blood sugar, cortisol, C-reactive protein

Experiencing weight stigma predicts *increased weight gain and obesity* over time *regardless of baseline BMI, age, race/ethnicity, and socioeconomic factors*

Bucchianeri et al., 2014; Hatzenbuehler et al., 2009; Koball & Carels, 2011; Madowitz et al., 2012; Puhl, Himmelstein, Watson, 2019; Puhl & Luedicke, 2012, Sutin et al., 2018; Wu & Berry, 2017. Daly et al., 2019; Himmelstein et al., 2015; Major et al., 2012; McCleary-Gaddy et al., 2019; Sutin et al., 201-Schafer & Ferraro. 2011; Schvey et al., 2014; Sutin et al., 2014; Tomiyama et al., 2014; Tsenkova et al., 201 Wu & Berry, 2017 Puhl et al., 2017; Jackson et al., 2014: Schvey et al., 2019; Sutin & Terracciano, 2013; Sutin et al., 2014; Quick et al., 2013



Person First Language

A 45 year old, obese male with diabetes and colon cancer...

A 45 year old, cancerous male with obesity and diabetes...

A 45 year old male with obesity, diabetes, and colon cancer...

- Put the person before the disability or diagnosis.
- Obesity is a disease. It doesn't define people

Language

Preferred

- Obesity
- Excess weight
- Overweight
- Weight issue or weight problem
- Higher weight
- Body mass index

Avoid

- Obese
- Morbidly obese
- Heavy
- Fat

Talk about weight objectively

- Tell me about your weight...Tell me about your child's weight...
- Is it ok if we talk about your weight for a moment?
- Let's look at your vital signs. Your BP is xx. I see your weight is up/down x pounds...

Non-judgmental intervention

- Do you need to address their weight at this moment?
- “Your weight may be contributing to your health issues”
- “We understand a lot more about weight. There are safe, effective treatment options. How can I help you get to the next step?”
- Approach obesity like any other diagnosis without judgement

Now what?

- Approach all patients with empathy and respect
- You can make a dramatic impact on helping patients
- **Remember that people are doing the best they can with the tools they have**

Patient Panel

